Anesthesia Incident Report

Basic Information

Age Range
21-30 years

Sex
Female

Weight
-- kg

Height
-- cm

Procedure Information

Type of Procedure
Elective

Surgical Branch
Gynecology

Procedure
DHC

ASA Grade
I

Incident Narrative Description

Equipment checked prior to surgery. After induction for GA with ETT and insertion of Veress needle, while attempting to insufflate, the insufflator malfunctioned. Surgery was cancelled and relaxant reversed and extubated.

Anesthesia Technique

Primary
> General Anesthesia
> ETT

Monitoring Used

ECG
Capnograph
Pulse Oximeter
NIBP
AGM
Temperature
ETT Cuff Pressure

Incident Demographics

Time of Incident
> Working hours

Place of Incident
> Operating room

Timing of Event
> On induction

Incident Detection
> Clinically by surgeon

Incident Relation
> To Surgery
> Multifactorial

Equipment Related

Gas supply problems
Others: Insufflator malfunction

Patient Safety

Others: Anesthetic delivered - surgery not done

Patient Outcome Category

Error, Harm
> Category E - An error occurred that may have contributed to or resulted in temporary harm to the patient and required intervention.

Reviewer's Comments

Review #1

Incident & Outcome

Malfuction of insufflator leading to cancellation of surgery post induction

Contributing Factors

Contributing factors for machine errors :
Nature or surgery : Emergency> Elective
Type of institution : stand alone > institutions
Consultant : Freelance > full time consultant.
Frequency of surgeries : less number of surgeries - more machine errors.
Communication and team : if in institution - a single point of contact - should be assigned the role of communicating machine errors to biomedical engineering department.

Root Cause & Prevention

Root cause and prevention:
1. Regular annual mainataince of machines.
2. Periodic calibration
3. Machine check including surgical instruments - laparoscopic/ robotic / cylinders- co2 etc before use.
4. If in institution - a single point of contact in a department should be assigned the role of communicating machine errors to biomedical engineering department.
Roles of this person :
* Documentation of machine , instrument errors.
* Communication with biomedical engerring department
* Uploading of nature of event to MvPI portal .
* Follow up of same.
* Intradepartment root cause analysis and documentation.
* Coordinating Interdepartmental committees (Anaesthesiologiolist , surgeons - ortho , surgery, gyneac , ent , opthal biomedical engineering) with meetings chaired by medical superintendent for discussion and analysis of anesthetic and surgical instruments errors.

Suggested Course of Action

Suggested course of action :
Protocol :
Step 1 .Medical college / tertiary care institution : Inform the institutional Biomedical engeneering department about the machine / instrument error.
Step 2 : Biomedical engenerring department of the institution should be registered with MDMC and the nature of instrument/ machine error should be registered through MvPI portal.
Step 3 : Root cause analysis will be done
Step 4: Interparty meeting with all stakeholders including manufacturers.
Step 5 :Report will be generated highlighting the Direction which can be either instruction to manufacturer or guidance to user

Alternatively :
If the institution is not registered with MDMC then the machine errors have to be reported to host nodal institution .
All the other steps remain the same .

Reference

https://adrmsipc.in/adrms/index.html